



Cottonwood Psychiatric Evaluation

Patient Name: Zahmoul , Nadia

Creator Staff Name: Turner, Deborah

Date Created: 04/02/2013

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Identifying Information:

Nadia is a 43-year-old female currently living in London but born in Morocco and has had time living in New York City as well. She is married and has two children, both girls, ages 7 and 5.

Referant:

Informants:

Chief Complaint:

Cocaine dependence.

History of Present Illness:

The patient states that last year during a very stressful time when she was moving "one more time" and her mother-in-law had cancer and she was caring for her, she states also stressors included relationship with husband and nanny, as well as poor sleep and her husband traveling quite a bit; she states that they were moving and her husband went away to Russia for business, she states that she began using cocaine. In the beginning she felt that this was helpful for her as she felt that it allowed access to trust herself and eliminate her self-doubt. She states prior to that she had "no access to what is strong inside". She states there was no more second-guessing, and she states she moved mountains in six months when she used cocaine, and she states that early on it did not control her but rather it was very helpful. She states that that did turn around and it began controlling her, which is why she has come into treatment to stop the use of cocaine. She states that she has had the help, through this time, of Professor Libby, who diagnosed her with severe depression and she was begun on Cymbalta, titrated up to 120 mg. She states that since that time she has had no more darkness. She states that she is a happy person, and she states "I've been controlling my moods ever since unless I'm expecting my period". She sees Professor Libby every Monday at 1 p.m. for the last year and feels this has been a very helpful relationship with her. She also has been seeing a psychoanalyst once per week, as well as a psychiatrist who specializes in addiction medicine. She feels that each of these relationships has been very helpful for her, however, at this time she feels the need to come inpatient for additional help and support in stopping use of cocaine.

Chemical Dependency History:

The patient states that she has used cigarettes, caffeine, some champagne, but not to excess; and most recently cocaine.

Past Psychiatric History:

As stated above, the patient does have several providers that she works with in London who she has developed strong relationships with and wishes to continue with them. Currently is on Cymbalta 120 mg. She states that the last time she was on 120 mg she did have some high energy and she was brought down to 90 mg, and very recently Professor Libby brought her dosage up to 120 again to combat increase in depressive symptoms.

Developmental History:

The patient was born in Casablanca, Morocco. She describes her mother as very manipulative and verbally abusive. She states that daily her brother was whipped with a horse whip. She states that she lived in fear constantly. She began drinking coffee at a young age and she would stay up until 3 or 4 a.m, and that was the only time of the day where she felt safe. She states she became #1 academically, and this became her identity. She states she had no access to her father and that her mother was in complete control. She states she started using cigarettes at 18. She went to London for a degree in Economics, and even there, parents were very controlling. They insisted that she be in

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her room by 7 p.m. and that they would call her every day to ensure this, and that if she was not, she would have to go back to Morocco. She complied with these rules and went and completed her degree. After completion of the degree, she was offered a job for which she was very excited. Her parents invited her back to Morocco for a visit prior to starting her job. She went back. She states her father asked her for her visa and her passport, which she gave him; he then locked them up in a safe and told her she was never going to leave Morocco again, and not leave their house until she was married. She then describes being kept in that home with constant verbal abuse. Throughout this time she states that she lived in a room that had no windows and no door: she called it "the pool room". When she lived in Morocco at 22 years old, she did get a job for Citibank - the American Bank, and she worked with the Moroccan government at that time on the federal budget. Her parents remained focused on her marriage, and she states "treated me like a dog". She states it was shameful to not be married. She repeated her pattern of being up until 4 or 5 a.m. and then go to work. She states that she was miserable. She went to New York on a training for her job, and she met someone there who she states she knew would be her husband. She states he was perfect for her because he was a type A and a go-getter and would not really ever get to know her. She states this was the only type of relationship she felt she could do, they married, and she states that that lack of intimacy has come back to be difficult for her in that she describes herself as feeling very lonely in her marriage. She worked for Lehman Brothers for 10 years. She was present for the 9-11 destruction. She states that **her marital relationship had poor communication** and that they made millions, but they fought a lot and were very lonely. She states that her in-laws' relationship with her husband was difficult. She describes her mother-in-law as loving and the first maternal relationship she has had. Her father-in-law was described as a narcissistic personality with no empathy and _____ demanding. She describes this as having a significant impact on her husband, who she feels "had his wings clipped" and had no ability to plan or be a part of a relationship. She states that they moved from New York to London, and she states that she did not want to move again and wanted to set some roots, and **her husband has been very negative towards her** and **has often been verbally abusive as well**. Her mother-in-law did have cancer and she did care for her, and she recently passed away.

Allergies:

See History and Physical.

Family History:

Unclear. Patient denies that her parents used any alcohol or drugs, however, there was significant abuse throughout the home. Family psychiatric history is largely unknown.

Social History:

Patient lives in London with her husband and two children. She has had a nanny over the years, which she has had what sounds like a strained relationship with.

Medical History+ Allergies:

See History and Physical.

Mental Status Exam**General Observations:**

Patient is alert and oriented, very intense demeanor. Seems psychomotor agitated.

Mood and Affect:

Mood is variable throughout interview. Affect is quite intense, crying at times, to being very calm at times, quite labile

Thought Content and Processes:

Thought processes are structured and logical at times, and tangential at others. No auditory hallucinations. No current suicidal or homicidal ideation, intention or plan.

Sensorium and Mental Capacity:

Insight seems to be fair. Judgment is fair as patient is currently seeking treatment for cocaine dependence.

Provisional DSM-IV Diagnoses: (This facility considers all Axis I diagnoses to be co-occurring)

<u>Axis:</u> Axis I	<u>Axis Code:</u> 309.81
<u>Axis Description</u> Posttraumatic Stress Disorder.	
<u>Axis:</u> Axis I	<u>Axis Code:</u> 304.20
<u>Axis Description</u> Cocaine Dependence.	
<u>Axis:</u> Axis II	<u>Axis Code:</u> 799.9
<u>Axis Description</u> Diagnosis Deferred on Axis II.	
<u>Axis:</u> Axis III	<u>Axis Code:</u> 000.0
<u>Axis Description</u> See H&P.	
<u>Axis:</u> Axis V	<u>Axis Code:</u>
<u>Axis Description</u> 40.	

Assessment:

Nadia is a 43-year-old female who appears approximately her stated age. Reports no social or familial supports. At home, she does describe caring for her children a great deal. She is very bright and accomplished. She does have significant traumatic history, and has experienced **significant symptoms of hypervigilance** and **high startle response**. She has had a great deal of difficulty sleeping throughout her life, and **she has had detachment and restricted** affect through most of her life as well. The patient did begin using cocaine recently, which in the beginning she felt was helpful for her in achieving a certain amount of self-confidence and accessing a part of herself that she did not know existed. She states that it did take control, however, and now she needs help in eliminating that from her life.

Recommendations and Initial Treatment Plan:

1. Admit to adult unit as patient is at severe risk of relapse if discharged at this time
2. Patient to begin work with therapist, group, individual, equine, consider **EMDR**, consider acupuncture.
3. Patient will be transferring care to Dr. Onate as she feels more comfortable with males.
4. Call in to Professor Libby in London to collaborate on the treatment plan.

Electronically authenticated by Deborah Turner M.D. on April 3, 2013 10:03 am